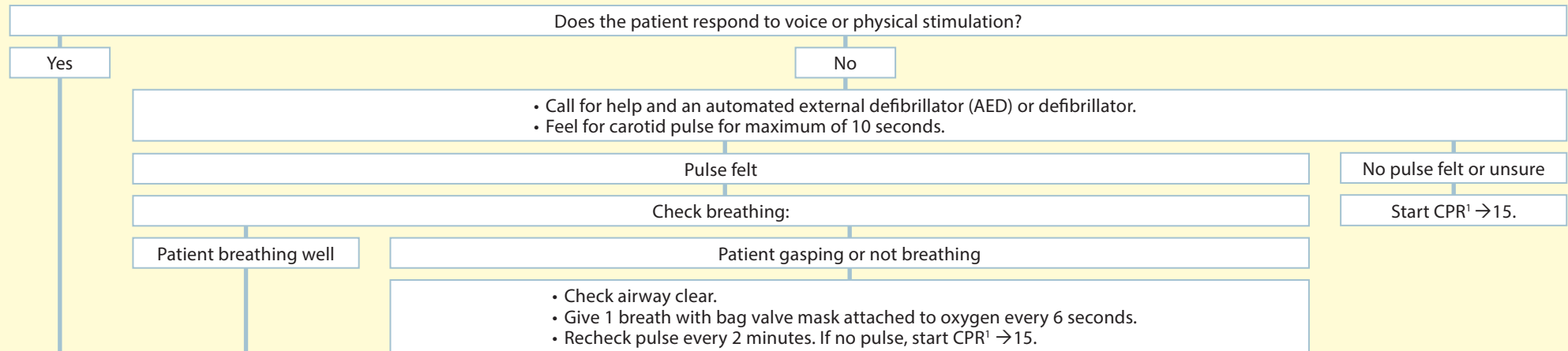


THE EMERGENCY PATIENT

Give urgent attention to the emergency patient:



Assess and manage airway, breathing, circulation and level of consciousness

Airway

- If airway obstructed (snoring, gurgling, noisy breathing), open with head-tilt and chin-lift. If injured, use jaw-thrust instead, keeping neck stable.
- Remove foreign bodies from mouth and suction fluids.
- If unconscious, insert oropharyngeal airway. If patient resists, gags or vomits, use lubricated nasopharyngeal airway instead.
- Intubate if unable to maintain airway with oro- or nasopharyngeal airway.

Breathing

- If difficulty breathing or oxygen saturation < 94%, give face mask oxygen.
- If respiratory rate < 9 or blue lips/tongue, connect bag valve mask to oxygen and slowly deliver each breath with the patient.
- Intubate if using bag valve mask and still difficulty breathing, oxygen saturation < 94% or blue lips/tongue.
- If sudden breathlessness, more resonant/decreased breath sounds/pain on 1 side, deviated trachea: **tension pneumothorax** likely:
 - Insert large bore cannula above 3rd rib in mid-clavicular line.
 - Arrange urgent chest tube.

Circulation

- Establish IV access.
- If pulse < 50 and unstable (BP < 90/60, decreased consciousness, chest pain or acute heart failure²): give **atropine** 0.5mg IV. Repeat every 3 minutes, up to a total of 3mg.
- If BP < 90/60, pulse ≥ 100 or heavy bleeding, give **sodium chloride 0.9%** 1L IV rapidly, repeat until systolic BP > 90. If known heart problem or severe infection suspected, give instead **sodium chloride 0.9%** 500mL IV over 30 minutes, repeat until systolic BP > 90. Continue 1L 6 hourly. Stop if breathing worsens.
- Stop bleeding: apply pressure and elevate limb. If bleeding still severe, apply tourniquet above injury.

Level of consciousness

- Assess Glasgow Coma Score (GCS):

Best motor response	Best verbal response	Eye opening
6 Obeys commands	5 Orientated	4 Spontaneous
5 Localises to pain	4 Confused	3 To voice
4 Withdraws from pain	3 Inappropriate words	2 To pain
3 Abnormal flexion to pain	2 Incomprehensible sounds	1 None
2 Extends to pain	1 None	
1 None		

- Add scores to give a single score out of 15:
 - If GCS ≤ 8, intubate patient.

Manage further and refer urgently:

- While awaiting transport, continue to assess and manage airway, breathing, circulation and level of consciousness.
- If decreased consciousness or vomiting, place in left lateral lying (recovery) position.
- If injured →18, if fitting/just had fit →19, if decreased consciousness →16, if burns →21, if bite/sting →22, if fever →24, if rash →67, if anaphylaxis →20.
- If other symptom, manage as on symptom page.

¹If the patient has a life-limiting illness, consider whether or not to proceed. If CPR not needed in the palliative care patient, address the dying patient's needs →172. ²Leg swelling, difficulty breathing which worsens on lying down/with effort.